Study Title: Optimising Chronic Pain Service Models in Primary Care: Addressing Medications associated with Dependence and Withdrawal Symptoms

Short title: OPSCAPE

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BACKGROUND

Chronic pain (lasting more than three months or more) is both a symptom of other conditions e.g., rheumatoid arthritis as well as a disease category itself (Chronic Primary Pain) (Treede et al., 2019). It is a contested diagnostic category with patients frequently experiencing trouble with diagnosis (Rosenberg, 2002; Werner & Malterud, 2003), stigma (Kempner, 2017; Perugino et al., 2022) and issues accessing health care support (Doebl et al., 2020).

Chronic pain is a significant public health issue in the United Kingdom, affecting around 28 million adults and placing a substantial burden on the healthcare system (Fayaz et al., 2016). Approximately 14% of adults living with chronic pain are considered to have high impact pain which significantly limits their quality of life (Fayaz et al., 2016) and leads to them frequently attending healthcare appointments (Blyth et al., 2004). Though specialist pain services exist and are promoted as a better care option for those suffering with chronic pain, there is huge variation in the services they offer, their availability, accessibility, and effectiveness (Price et al., 2019). This variation reflects existing societal inequities and inequalities with services less available where the need is highest (Jain et al., 2023; Versus Arthritis, 2021).

The focus of many of these medically led secondary and tertiary level services is on providing pain relieving and/or modifying medications and procedures (Price et al., 2019). This can be inadequate, given that those experiencing chronic pain often have multi morbid concerns with low mood, pain related distress, poor physical function, and decreased participation in civic and social life (Breivik et al., 2006). Models of multidisciplinary pain management including input from e.g., physiotherapists, psychologists and occupational therapists are recommended (British Pain Society, 2013) and for those who attend, offer good outcomes across pain intensity, disability and psychological functioning (Wilson, 2017). Along with the low numbers of these services available in England (National Pain Audit, 2012), eligibility and attrition remain an ongoing concern, with many patients being unable to benefit (Knight et al., 2019).

Primary care plays a crucial role in managing chronic pain, as it is often the first (and for some the only) point of contact for patients seeking relief and support (Smith et al., 1999). Several clinical guidelines have been produced which are intended to support non-specialist practitioners (including General Practitioners GPs) in delivering chronic pain work in the absence of comprehensive training in pain in the undergraduate medical curriculum e.g., (Blair et al., 2014; National Institute for Health and Care Excellence, 2021). The response from primary care to the problem of chronic pain has been hampered in part due to the increasing demand and the challenge of a reduced number of medicines considered effective for this population.

Medicines used to relieve or modify the pain experience include simple analgesics like paracetamol and non-steroidal anti-inflammatory drugs (NSAIDs), opioids, benzodiazepines, antidepressants and anti-convulsant drugs. Though helpful in some acute pain conditions, there is uncertainty about the comparative effectiveness and safety (Enke et al., 2018; Ferreira et al., 2023; Wewege et al., 2023) and, they all have limited utility in chronic pain (Els et al., 2017a; Enke et al., 2018). Attention on the opioid crises in many countries in the Global North stemming in part from over prescription of opioids for pain, has led to reticence and caution in continuing prescribing opioids, even for post-surgical acute pain (Levy et al., 2020). In the UK, prescribing patterns for opioids, suggest that GPs in areas of higher socioeconomic deprivation prescribe higher quantities and doses of opioids (Curtis et al., 2019; Mordecai et al., 2018), perhaps reflecting the inequitable distribution of pain services previously described (Jain et al., 2023).

These broader concerns are coupled with evidence that many of these medicines are 1) subject to illicit use and diversion; 2) procured through legal prescriptions (Advisory Council on the Misuse of Drugs,

2016); 3) associated with withdrawal and addiction (Els et al., 2017b; Hojsted et al., 2013; Taylor et al., 2019) and; 4) contribute to preventable deaths (France et al., 2023). Clinical practice guidelines used extensively in primary care now advise against starting new prescriptions (National Institute for Health and Care Excellence, 2021), supporting people with withdrawing and tapering off medicines (NHS England, 2023), and offering alternatives to support people living with chronic pain (Mills & Smith, 2022; Smith et al., 2014).

There is a growing recognition that current **service models in primary care** may not adequately address the complex needs of individuals with chronic pain, particularly regarding medications associated with dependence and withdrawal symptoms. The primary care response to the limited utility of pharmacological approaches to managing complex and contested chronic pain has been varied. For example, in a randomised controlled exploratory trial, Bruhn at al (2013) explored pharmacist-led management of chronic pain in primary care, comparing whether having a pharmacist review (with or without prescribing) had an impact on quality of life, pain intensity, mental health indices and healthcare utility when compared to usual care and GP follow up. They found that the service model was acceptable to both patients and most professionals, and that this model of care could lead to improvements in pain and other measures, though further study was required. Sandhu et al. (2023) investigated whether a multicomponent, group-based, self-management intervention reduced opioid use and improved pain-related disability compared with usual care in people with chronic pain. They found that the intervention led to a significant reduction in opioid use but had no effect on pain interference in daily activities.

Opioid reduction alone, in the absence of meaningful reduction in pain intensity or improvements in function and quality of life, is considered by some people living with pain as harmful and undesirable (Ljungvall et al., 2020). As such, it may be that applying a one-size-fits all to reducing, tapering, and stopping certain pain medicines could contribute to harm (e.g., being unable to work) from a patient's perspective. People seeking support for antidepressant withdrawal - frequently used for pain see e.g., (Ferreira et al., 2023)- reported that doctors' advice was often unhelpful, they didn't recognise symptoms of withdrawal and suggested tapering schedules that were too rapid (Read et al., 2023).

Outside experimental research study designs, evaluation of **real-life primary care led initiatives** show some promise in more personalised approaches to caring for people living with chronic pain and seeking support with medicines associated with dependence and withdrawal. Stellman et al (2022) reported on a new primary care led, integrated model of personalised care for people with fibromyalgia (a chronic pain condition) that allowed GPs to have a longer initial consultation appointment, tailored follow up and support of a multidisciplinary team including secondary care pain specialists. In their evaluation, they found that primary care staff felt supported and upskilled in managing this cohort of patients, that patients reported increased understanding of their symptoms, and improvement in their symptoms and confidence in managing the condition.

Whilst anecdotally and in the grey literature, there are case studies of exemplary primary care led initiatives for chronic pain and medicines associated with dependence and withdrawal symptoms for chronic pain, little is known and understood as to how they achieve their impact. Therefore, there is a need to understand and thus optimise chronic pain service models in the UK primary care setting including, but not exclusive to their prescribing practices, monitoring procedures, patient education and involvement, approach to risk and safety and involvement of plural providers.

Doing so has the potential to improved patient desired outcomes through enhancing pain care strategies, including non-pharmacological interventions and community interventions, can reduce reliance on medications and minimise the risks of dependence and withdrawal symptoms. Optimal management of chronic pain in primary care can lead to a decrease in accident and emergency visits, hospital admissions,

and medication-related complications, resulting in cost savings for the healthcare system. By addressing the specific needs and concerns of patients with chronic pain, primary care providers can establish stronger therapeutic alliances and enhance patient satisfaction.

Based on the background and rationale outlined above, this project aims to optimise chronic pain service models in primary care in the UK, specifically focusing on addressing medications associated with dependence and withdrawal symptoms. By synthesising the research and grey literature, mapping relevant policy and guidance and, conducting three in-depth ethnographic case studies, the project seeks to enhance provider practices, ensure more personalised care for individuals with chronic pain in primary care and in doing so potentially improve patient outcomes in the long term.

THEORETICAL BACKGROUND

This study is informed by Institutional Ethnography (IE), a sociological approach to inquiry that uses everyday experience to examine social relations and social institutions that structure people's lives (Smith, 2005, 2006). An IE lens will enable us to map the relations that influence and coordinate people's lives (both patients and practitioners) within institutions as they seek to live with (and support) chronic pain. We are interested in how socially organised institutions within the provider networks of primary care (e.g., Primary Care Networks (PCNs) and Integrated Care Boards (ICBs)) conduct the work of caring for people with chronic pain and how they address medicines associated with dependence and withdrawal symptoms. IE additionally, allows us to focus on intersectional issues, such as racism, which are known to lead to inequitable experiences of pain care (Macgregor et al., 2023), in part due to specific efforts in IE to understand and foreground otherwise invisible social relations which contribute to marginalisation of some groups (Webster et al., 2023). An IE approach will allow this work to expand on more descriptive reports (e.g.,Smith et al., 2014; Smith et al., 1999) of what constitutive elements form exemplar primary care-led service models for chronic pain.

RESEARCH QUESTIONS, AIMS AND OBJECTIVES

This research aims to examine how primary care-led exemplar service models navigate the inherent tensions between the risks and benefits associated with prescribed medications linked to dependence and withdrawal symptoms in the context of managing chronic pain asking the following **research questions**:

1) How is risk and harm understood in relation to medicines associated with dependence and withdrawal symptoms within the context of chronic pain management?
2) How do primary care-led exemplar service models deliver care for people with chronic pain?
3) How can people with chronic pain who are prescribed medicines associated with dependence and withdrawal symptoms be better supported in policy and practice?

Study aims and objectives are set out in the table below.

Aims	Objectives
1. To identify and describe what constitutes risk and harm in relation to medicines associated with dependence and withdrawal symptoms within the context of chronic pain care.	a) to conduct and report on a systematic hermeneutic literature review synthesising a large and diverse literature on what risk and harm *is*, and how different meanings and assumptions affect prescribing practices in chronic pain.
2. To understand how primary care-led exemplar service models deliver care for people with chronic pain (including those prescribed medicines associated with dependence and withdrawal symptoms).	b) to conduct three diverse in-depth ethnographic case studies of primary care-led exemplar service models for chronic pain, to highlight the ways in which pain care is proposed, supported, undertaken, and experienced by patients, peers, and professionals.
3. To understand how the policy and organisational context shapes support for people with chronic pain who are prescribed medicines associated with dependence and withdrawal symptoms.	c) To identify and analyse up to 10 policy documents policy documents relating to self-management and chronic pain and to identify the extent to which current policy supports those who have typically been excluded from pain management services.
2. To produce a typology of primary care-led services that constitute an exemplar for supporting people with chronic pain who are prescribed medicines associated with dependence and withdrawal symptoms.	d) To present research findings and develop with stakeholders, in 3-4 workshops, a freely available typology of exemplar primary care-led services for supporting people with chronic pain who are prescribed medicines associated with dependence and withdrawal symptoms.

STUDY SETTINGS

We have purposively identified three potential case study sites where will focus this study, from their work being presented as exemplar at academic and practice meetings in both primary care and pain management, our team's involvement in horizon scanning and strategic leadership and, existing relationships formed in the course of clinical work. We have made informal approaches to two (out of three) sites in preparation for this application and secured their agreement in principle to be case study sites (subject to funding and approvals). Our study sites represent both *collective* (including three contrasting service models) and *instrumental* (exemplars of primary care-led service delivery for chronic pain) case studies to allow us to study the complex social relations involved in delivering and receiving

care for chronic pain in real life contexts (Crowe et al., 2011; Stake, 1995).

Site 1

GP practice, with ~8,500 patients on the south coast of England leading design, delivery, scale and spread of a complex multi-faceted intervention for chronic pain in adults for the PCN ~550,000. The components include (but are not exclusive to patient and practitioner education, peer support and social prescribing. The intervention is delivered by different primary care professionals along with a patient with lived experience.

Site 2

Multi professional consortium including social care, multiple health providers, specialist pain providers, voluntary and community partners iteratively co-designing (and redesigning) with local patients a different primary care-led service model of pain care to an inner-city urban population with high levels of deprivation in London.

Site 3

To be determined. We will seek support for recruitment of the third site from the Clinical Research Network (CRN) as a portfolio study. We will focus on approaching areas with high prescriptions (compared to national averages on openprescribing.net) of medicines associated with dependence and withdrawal for chronic pain, to achieve geographical spread (likely the Northeast or Northwest of England).

DATA COLLECTION

IE as an approach employs the typical ethnographic methods of generating data of interviews, observations, and documents (both textual and pictorial). We will conduct ethnographic field work at each site over a 5-6 day period for the first visit as a starting point (standpoint) from which to anchor the inquiry (Webster et al., 2015), and another 5-6 day period up to 3 months later, allowing us to engage with longitudinal perspectives from each site. Once relevant approvals have been secured, we will arrange a formal site visit (each site) to meet the practice manager and stakeholders (as identified by our initial contact). The site visit will allow the research team to outline the nature of our work and to begin meeting with key informants. Further identification of texts and informants is not pre-determined but will be guided by the process of inquiry. Based on available literature on similar IE informed research, we anticipate recruiting up to six patient participants and up to 12 participants from the wider provider networks, per site (total = 54). We will use snowball sampling and staff insight to identify, during our week long visits, which patients meet the eligibility criteria to be invited to participate. Adults aged 18 years and above, who self-identify as having chronic pain (pain for more than 3 months), and/or who have a diagnosis of chronic pain in the Electronic Health Record will be eligible. There will be no English language requirement. Guided by local service provision, we will offer to arrange interpreters in participant's chosen language to facilitate an interview.

Wherever possible, we will conduct interviews with care providers and patients in and around the time of a care episode (e.g., a medications review) we have observed. We will seek and secure written informed consent before each research encounter with participants. Interviews with participants will be audio recorded (with consent). We will invite participants to share relevant documents that they use in negotiating care (patient participants) or making decisions to offer care (provider participants). Participants will be offered a go-along interview with one of the researchers. In **go-along interviews**, researchers accompany participants in their everyday lives and are a way of gaining insight into local, contextual and situated interactions (Carpiano, 2009).

We will use snowball sampling, asking those already participating in the study to discuss the study with their

contacts and (if they secure interest from them) advising their contacts to make themselves known to the research team if they think would be interested in hearing about the study and consider participating.

The researchers in the field will make contemporaneous field notes to record observations, reflections, and analytic memos throughout the research process.

Data management

This is a qualitative study, involving collection of audio-recordings, documents and other texts and transcripts of interviews and field notes. A data log will be set up (using Excel) to log details of each piece of data (including participant ID, data type, date collected and by whom) and connections between them (interview and field notes from each individual kept together). All data will de-identified with raw data stored on a dedicated area (accessible only to the research team) on a secure server at the University of Oxford along with a 'key' (i.e., a list of ID numbers and names) which connects the identifiable and de-identified data. Potentially identifiable material will not be used in published or publicly accessible outputs unless express written consent has been given (e.g., permission to use direct quotes will be sought).

Data collection and storage will follow the University policy on Data Quality and Data Quality Assurance, Management of Research Data and Records and Information Security We will also follow Nuffield Department of Primary Care Health Sciences Information Governance framework, which sets out procedures and structures to comply with the University regulations using the Department information systems. In accordance with these policies, all confidential and personal data (including names, addresses, telephone numbers) will be kept on the department's secure network and only accessible to members of the department directly involved in the project.

DATA ANALYSIS

All audio recordings of interviews will be professionally transcribed including all the words spoken by interviewee and interviewer, with attention to significant hesitations, exclamations of emotion (e.g., laughter) and long pauses. We will review transcripts together during regular team meetings, along with field notes and analytic memos made during observations. All members of the research team will be involved in data analysis. This first step of familiarisation will allow us to become immersed in the data as part of the analytic process. We will then begin to bring together *first order* groupings of ideas (similar to codes), attending to sequences of action related to our research questions as guided by the IE approach (Smith, 2006). These initial groupings will be informed iteratively by our insights from the hermeneutic literature review and will in turn steer further data collection in our case study sites.

We will then move to second order interpretations where the analytic focus will be on the social organisation of activities surfaced in first order analyses and referenced in dominant discourses, texts, and institutional structures. We will draw on relevant theory to further explain our findings, illustrating the connection between theory and how practices are done through quotations from the data.

We will report on our findings by integrating data from the hermeneutic literature review and policy analysis with insights from the aggregated case studies and presenting a 'typology of primary care-led service models for chronic pain' to assembled stakeholders in dissemination workshops at each site. Participants at each workshop will be invited to contribute further and refine the resultant typology.

This approach to data analysis, grounded in an IE approach, will allow us to build a rich, theoretically informed picture of support for people prescribed medicines associated with dependence and withdrawal symptoms for chronic pain from multiple perspectives whilst being sensitive to local context and multiple interdependencies.

ETHICAL CONSIDERATIONS

The proposed research study involves human participants (NHS staff and patients) and as such is likely to encounter ethical issues and will require HRA/Ethics approval before study activities begin. The protocol, informed consent form, participant information sheet and any proposed advertising material will be submitted to an appropriate Research Ethics Committee (REC), HRA (where required), and host institution(s) for written approval.

The methods proposed of in-depth ethnographic observation, interviews, and go-along interviews, and accessing electronic health records may be intrusive to participants and raise concerns about privacy, confidentiality, and anonymity. We will ensure that this study is conducted in accordance with relevant regulations and with Good Clinical Practice. Written informed consent will be a precondition for inclusion of individuals within the study. We will arrange to translate materials to select languages if/when we receive an expression of interest to participate by people who do not speak English (and have requested the budget to facilitate that). Care will be taken to provide sufficient time and information to enable informed decisions about participation and to avoid - as far as possible - placing obligations on participants to contribute to the study.

All participants will remain anonymous and the study sites will not be named in any study materials or reports, unless express consent is given to deanonymize e.g. case study site location. Participants will be identified only by pseudonym on study documents and any electronic database. All documents will be stored securely and only accessible by study staff and authorised personnel. The study will comply with the data protection regulation.

Should a patient raise a potential complaint about their care during the course of the research, the researcher would signpost them to the relevant processes to follow for their respective organisation. The research team would not be involved in assessing or resolving complaints.

We propose following a small group of patient participants within their own communities and across settings e.g., health care visits. Participants will always make the final decision as to the researcher's presence in these settings. The overarching principle will be guided by a mature and productive relationship with the participants, enabling on-going negotiation about their participation in the study and permission to withdraw from the study. Ethical issues will be a recurring agenda item in research team meetings and with our Research Advisory Group and given as much discussion time as needed.

GOVERNANCE AND PROJECT MANAGEMENT

We will convene a Research Advisory Group with membership from General Practitioners with an interest in chronic pain, Clinical Pharmacist with responsibilities for opioid stewardship and harm reduction, Policy Advisor to voluntary organisation for pain, people with lived experience of pain, Consultant in Pain Medicine researching medicines associated with dependence and withdrawal. The research advisory group will be chaired by our public collaborator and person with lived experience of pain and medicines

associated with dependence and withdrawal. All members have expressed a commitment in principle and if funded, we will extend formal invitations. This group will meet at least every quarter and provide oversight to the project.

The research team will meet monthly including our public collaborator working to deliver the project milestones and objectives. Dr Walumbe will have overall responsibility for the project delivery in her first lead PI role having been CI for her doctoral work and co-app on two other recent projects. Dr Walumbe and Dr Finlay will be the main researchers co-leading on the ethnographic case studies. Dr Finlay will provide overall guidance for the team on Institutional Ethnography. Administration and operations will be supported by Charlotte Grant Thompson.

DISSEMINATION AND OUTPUTS

Our dissemination and impact strategy includes the following outputs for academic, patient/pubic and NHS/practitioner audiences.

1. Academic audience, with the aim of disseminating key findings, engaging academic researchers working in chronic pain and primary care.

- **Open-access academic paper** - in a high impact, peer-reviewed international journal (e.g. PAIN, Sociology of Health & Illness).
- **Conference presentations** - at national/international events including Health Services Research UK Annual conference.
- **Up to 54 audio-recorded interviews -** available for review and/or secondary analysis.

2. NHS practitioners, decision-makers and professional groups (e.g. British Pain Society), with the aim of maximizing coverage and potential utility to the NHS, and informing quality standards for primary care-led chronic pain service models.

- A **project briefing paper** (following the 1-3-25 format, ensuring an accessible summary of research findings backed up by detailed analysis and discussion) outlining the typology of primary care-led services for chronic pain- to be sent to all NHS staff (as well as those living with chronic pain) participating in the study and including a link to the project website.

3. Patients and the public, with the aim of sharing emerging findings, enriching the research process, and remaining connected with the social context in which the lived experience of chronic pain exists.

- Ad hoc **informal discussion** about the research and emerging findings – engaging with local pain support group meetings, voluntary/third sector organization meetings supporting chronic pain (e.g. Pain Concern, Arthritis Action).
- **Online commentary** about the research - via online blogs and/or newsletters (e.g. The Conversation - theconversation.com/uk).

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